HSRA State Prioritization Ranking 2026

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Purpose

This document replaces the earlier loose state-targeting logic with a more defensible rank-ordering model for HSRA / PopHealthMap outreach. It uses per-state public evidence where available, separates `problem fit` from `messaging responsiveness`, and avoids pretending we can predict exact reply rates from public signals alone. It now also incorporates burden-led differentiators surfaced in the deeper state framework review, especially rural hospital vulnerability, cancer-screening disparity pressure, and Medicaid non-expansion urgency.

Ranked States For Active Pursuit

Tier A - Highest priority now

These states combine strong public problem-fit signals with credible evidence that HSRA-style messaging will land well.

Rank	State	Why it ranks here	Confidence
1	Tennessee	Strongest commercial anchor, existing internal traction, credible Tennessee-first proof story, solid RHT funding, and best current conversion path from outreach into real meetings.	High
2	Texas	Highest FY2026 RHT award (`$281.3M`), explicit `Rural Texas Strong` county-outcome framing in current research, strong hospital CHNA analogs, and a very clear rural-transformation message lane.	High
3	Colorado	Official `Colorado EnviroScreen 2.0` , clear post-EJScreen replacement relevance, solid RHT funding, and a strong tract-level burden-screening story that maps directly to HSRA.	High

Rank	State	Why it ranks here	Confidence
4	Pennsylvania	Official `PA HEAT` plus a state SHA dashboard, strong public county-level indicator infrastructure, and clear planning / disparity / state-assessment message lanes.	High
5	Connecticut	Official health-equity dashboards and official CT EJScreen-style tooling create both strong fit and strong evidence that the state recognizes configured analytics products in this category.	High
6	North Carolina	Strong CHNA / CHA collaboration evidence, clear local-health and hospital planning relevance, solid RHT award, and a practical planning-oriented entry path.	Medium
7	Minnesota	Official health-equity data infrastructure, county public-health profiles, and environmental-justice concern mapping make the integrated HSRA story easier to explain.	High
8	New York	Strong public health-indicator and Prevention Agenda infrastructure, annual hospital reporting pressure noted in research, and multiple health-equity / community indicator tools.	High
9	Kentucky	Official health equity dashboard plus strong RHT funding create a good combination of visible problem fit and message receptivity, even though the state lacks the same breadth of signals as Tier A leaders above it.	Medium
10	Oregon	Official SHA / SHIP and county-level indicator infrastructure create a strong planning message lane; Oregon is slightly lower only because the public evidence is more planning-heavy than funding- or EJ-heavy.	Medium

Tier B - Strong fast followers

These states have meaningful public signals, but the evidence base is narrower or more one-dimensional than Tier A.

Rank	State	Why it ranks here	Confidence
11	Maryland	Maryland now moves up because the deeper review surfaced a rare combination of rural-health strategic planning, active cancer-control planning, and a state EJ analog, which makes both problem fit and message clarity stronger than the earlier draft captured.	High
12	Alabama	Alabama remains a strong fast follower because rural burden, non-expansion pressure, PHAB activity, and planning fit now look more substantial than the earlier evidence sweep showed.	Medium
13	Washington	Public health-disparities / location-based tools and official equity-related dashboards exist, but the exact HSRA conversion path is still less direct than in Colorado, Pennsylvania, or Maryland.	Medium
14	California	CalEnviroScreen is a major tract-level analog and RHT funding is strong, but the likely market is more crowded and the message may need sharper differentiation against existing mapping tools.	Medium
15	Georgia	Georgia moves into the fast-follower group because rural burden, non-expansion pressure, and a recent cancer-control plan create stronger demand-side evidence than was reflected in the earlier ranking.	Medium
16	Mississippi	Mississippi has one of the strongest burden-led cases in the new framework; it stays below the top group only because access-path and message-responsiveness evidence are still thinner than for Tier A states.	Medium
17	Arkansas	Arkansas rises on rural hospital vulnerability, PHAB activity, and strong unmet-need evidence, even though the buyer-path evidence remains less developed than in the top planning-heavy states.	Medium

Rank	State	Why it ranks here	Confidence
18	Vermont	Vermont's combination of extreme rurality, public health-equity dashboards, and EJ-tool development makes it a credible upper-middle target despite a smaller commercial surface area.	Medium
19	Nebraska	Strong RHT funding plus official community health dashboards and SHA / SHIP logic keep Nebraska in the fast-follower band.	Medium
20	Kansas	Kansas enters the ranked fast-follower group because rural burden, hospital-vulnerability evidence, PHAB activity, and non-expansion pressure create a clearer need case than several previously listed states.	Medium

Tier C - Broad-launch states with narrower differentiated evidence so far

These states remain in all-state launch coverage and may move up quickly if better state-specific evidence or real response signals emerge.

Rank Band	States	Current read
21-30	Alaska, Oklahoma, Missouri, New Mexico, Florida, Iowa, Wisconsin, Wyoming, Nevada, Montana	Respectable burden or RHT signals exist in several cases, but the current record still offers less proof of ready buyer-paths than the Tier A and Tier B states.
31-40	Illinois, New Jersey, Indiana, Louisiana, New Hampshire, South Carolina, West Virginia, Utah, Virginia, Idaho	These states remain plausible fits, but the current public record is either narrow, one-dimensional, or not yet strong enough to push them into the fast-follower sequence.
41-50	Arizona, Delaware, Hawaii, Maine, Massachusetts, Michigan, North Dakota, Ohio, Rhode Island, South Dakota	Current evidence sweep found fewer differentiating public signals relative to other states; keep them in launch and nurture unless campaign data changes their standing.

Important Interpretation Rule

This ranking is **not** saying lower-ranked states do not matter. It is saying:

- all 50 states should still receive launch messaging
- the top-ranked states deserve the fastest research, strongest state-specific message tailoring, and earliest live discovery effort
- lower-ranked states should stay in launch coverage and nurture until new evidence or real campaign responses change the picture

Methodology

Two-axis model

Following Oracle review, each state is assessed on two separate dimensions:

1. `Problem fit` - does the state show visible public evidence of the kind of planning, screening, disparity, or prioritization problem HSRA solves?
2. `Messaging responsiveness` - does the state show public evidence that HSRA-style messaging will look familiar, timely, and actionable to likely buyers?

Scoring rubric

Each sub-signal is scored coarsely to avoid fake precision.

Score	Meaning
0	no meaningful differentiating public signal found in the current sweep
1	weak or indirect signal
2	one strong public signal or multiple moderate signals
3	multiple strong and recent public signals

Problem fit components

Component	Weight	Evidence used
Rural burden and coverage pressure	25%	rural hospital vulnerability, rural outcome burden, Medicaid non-expansion pressure
Public-health planning pressure	20%	SHA, SHIP, CHA, CHIP, PHAB, CHNA, county indicator infrastructure
EJ / environmental-health screening need	20%	official state EnviroScreen-style tools or visible tract-level EJ logic
Cancer / screening / disparity targeting fit	20%	public cancer-control, screening, or disparity-targeting signal
RHT intensity	15%	FY2026 CMS Rural Health Transformation award amount

Messaging responsiveness components

Component	Weight	Evidence used
State dashboard / analytics maturity	35%	official state dashboards, public indicator portals, health equity tools
Adjacent procurement / configured analytics evidence	35%	public dashboard or analytics engagements, CHNA/CHA collaboration evidence
Message-lane clarity	20%	whether a clear state-specific message lane is obvious from public evidence
Access-path clarity	10%	whether likely account owners are easy to identify from public structures

RHT intensity buckets

Because all 50 states received RHT awards, this is a differentiator only by degree.

FY26 award amount	RHT intensity score
`>= $220M`	3

FY26 award amount	RHT intensity score
`$200M–$219.9M`	2
`$180M–$199.9M`	1
`< $180M`	0

Confidence labels

Confidence	Meaning
High	3 or more recent sources across multiple signal types
Medium	2 useful sources or mixed evidence strength
Low	mostly baseline RHT signal or limited state-specific evidence

Evidence Base Used

This ranking uses the following evidence categories already gathered in repo research plus targeted state-source checks:

- official CMS FY2026 RHT award amounts for all 50 states
- rural hospital vulnerability and rural burden signals from Chartis and related public-health reference sources
- official or public state EJ / burden-screening tools: California, Colorado, Connecticut, Illinois, Maryland, Minnesota, New Jersey, Washington, and related state materials for New York
- official state health-equity or disparity dashboards: Connecticut, Kentucky, Pennsylvania, New York, Minnesota, Washington
- official SHA / SHIP / county-indicator or state assessment infrastructure: Pennsylvania, Oregon, Minnesota, Nebraska, New York
- public CHNA / CHA or analytics-procurement analogs: North Carolina, Connecticut, Kentucky, New York
- public cancer-control / screening-gap and Medicaid-expansion-pressure evidence from the deeper state framework review
- internal commercial anchor evidence for Tennessee
- `docs/research/hsra/hsra-state-outreach-priority-framework-2026.md`

Why Some States Rank Above Others

The strongest differentiators found so far

1. `Rural burden, hospital vulnerability, and coverage-gap pressure`
   - strongest examples: Mississippi, Arkansas, Alabama, Kansas, Tennessee, Texas, Georgia
2. `Official state EJ / burden-screening tools`
   - strongest examples: California, Colorado, Connecticut, Illinois, Maryland, Minnesota, New Jersey, Washington
3. `Official state health-equity dashboards or disparity data tools`
   - strongest examples: Connecticut, Kentucky, Pennsylvania, New York, Minnesota, Washington
4. `Official SHA / SHIP / county indicator infrastructure`
   - strongest examples: Pennsylvania, Oregon, Minnesota, Nebraska, New York
5. `Explicit public analytics / CHNA / assessment analogs`
   - strongest examples: North Carolina, Connecticut, Kentucky, New York
6. `Cancer-control and screening-gap urgency`
   - strongest examples: Maryland, Georgia, Mississippi, Kentucky, New Mexico
7. `Exceptional RHT funding intensity`
   - strongest examples: Texas, Alaska, California, Montana, Oklahoma, Kansas

Why Texas and Colorado score especially well

- `Texas` combines the strongest RHT award amount with an explicit public research analog around county-level outcome prediction and strong hospital planning evidence.
- `Colorado` has unusually clear official evidence of tract-level environmental burden scoring already being used to direct funding and resources.

Why Pennsylvania and Connecticut moved up

- `Pennsylvania` has both official health-equity analysis tools and official SHA dashboard infrastructure, which raises both problem fit and message receptivity.
- `Connecticut` has public health-equity dashboards plus an official EJ screening tool, which makes HSRA easier to explain as a configured, state-first analytics layer.

Why Maryland, Georgia, Mississippi, and Vermont now matter more

- `Maryland` now looks stronger because it combines a state EJ analog, a current rural-health strategic plan, and an active cancer-control planning lane.
- `Georgia` gains importance because non-expansion pressure, rural burden, and a recent cancer-control plan create a real unmet-need story even if the buyer path is not yet as clear.
- `Mississippi` remains a burden-led challenger with unusually strong hospital-vulnerability and disparity signals; it is now hard to justify keeping it out of the fast-follower set.
- `Vermont` has a smaller market surface area, but its extreme rurality plus recent health-equity and EJ-tool work make it stronger than a generic small-state assumption would suggest.

50-State Working Matrix

This appendix keeps all 50 states in view. For many states, the current sweep found only baseline RHT evidence plus general national problem-shape alignment. That does **not** mean they are bad targets; it means the public differentiators are weaker today.

State	FY26 RHT Award	Extra state-specific public signals surfaced in current sweep	Initial tier	Confidence
Alabama	$203.4M	rural hospital burden, non-expansion pressure, and PHAB / planning activity	B	Medium
Alaska	$272.2M	no major differentiator surfaced beyond high RHT amount	C	Low
Arizona	$167.0M	no major differentiator surfaced in current sweep	C	Low
Arkansas	$208.8M	extreme rural hospital vulnerability; PHAB activity; strong burden-led need case	B	Medium
California	$233.6M	CalEnviroScreen official analog	B	Medium
Colorado	$200.1M	Colorado EnviroScreen 2.0 official analog	A	High

State	FY26 RHT Award	Extra state-specific public signals surfaced in current sweep	Initial tier	Confidence
Connecticut	$154.2M	official health equity dashboards; official EJ screening tool	A	High
Delaware	$157.4M	no major differentiator surfaced in current sweep	C	Low
Florida	$209.9M	no major differentiator surfaced in current sweep	C	Low
Georgia	$218.9M	rural burden; non-expansion pressure; recent cancer-control planning activity	B	Medium
Hawaii	$188.9M	no major differentiator surfaced in current sweep	C	Low
Idaho	$186.0M	plausible rural-burden fit but thinner differentiated evidence than upper tiers	C	Low
Illinois	$193.4M	official EJ Start tool, but thinner burden and planning evidence than current fast followers	C	Medium
Indiana	$206.9M	no major differentiator surfaced in current sweep	C	Low
Iowa	$209.0M	no major differentiator surfaced in current sweep	C	Low
Kansas	$221.9M	rural hospital vulnerability, PHAB activity, and non-expansion pressure	B	Medium
Kentucky	$212.9M	official health equity dashboard	A	Medium
Louisiana	$208.4M	strong burden-led fit but weaker buyer-path evidence in current sweep	C	Low-Medium
Maine	$190.0M	no major differentiator surfaced in current sweep	C	Low

State	FY26 RHT Award	Extra state-specific public signals surfaced in current sweep	Initial tier	Confidence
Maryland	$168.2M	official MDEnviroScreen analog; rural health strategic plan; active cancer-control planning	B	High
Massachusetts	$162.0M	health-equity and EJ infrastructure exist, but current HSRA path is less differentiated than in top tiers	C	Medium
Michigan	$173.1M	no major differentiator surfaced in current sweep	C	Low
Minnesota	$193.1M	official health-equity/statistics resources; county profiles; EJ concern mapping	A	High
Mississippi	$205.9M	extreme burden-led case: hospital vulnerability, poor health outcomes, equity-pressure signals	B	Medium
Missouri	$216.3M	no major differentiator surfaced in current sweep	C	Low
Montana	$233.5M	strong rural-fit logic in current HSRA docs, but thinner public buyer-path evidence than current fast followers	C	Low-Medium
Nebraska	$218.5M	official dashboard / SHA / county indicator infrastructure	B	Medium
Nevada	$179.9M	state planning / modernization fit cited in current HSRA docs, but less differentiated burden evidence than updated fast followers	C	Low-Medium
New Hampshire	$204.0M	no major differentiator surfaced in current sweep	C	Low
New Jersey	$147.3M	official EJ mapping / protection tool, but lower burden intensity and	C	Medium

State	FY26 RHT Award	Extra state-specific public signals surfaced in current sweep	Initial tier	Confidence
		narrower current buyer-path evidence		
New Mexico	$211.5M	no major differentiator surfaced in current sweep	C	Low
New York	$212.1M	CHIRS / Prevention Agenda / health equity and reporting infrastructure	A	High
North Carolina	$213.0M	CHNA / CHA collaboration evidence; local planning fit	A	Medium
North Dakota	$198.9M	no major differentiator surfaced in current sweep	C	Low
Ohio	$202.0M	meaningful planning and EJ signals exist, but the HSRA-specific message lane remains less distinctive than top-ranked states	C	Medium
Oklahoma	$223.5M	no major differentiator surfaced beyond strong RHT amount	C	Low
Oregon	$197.3M	official SHA / SHIP and county indicator infrastructure	A	Medium
Pennsylvania	$193.3M	PA HEAT + SHA dashboard + county-level planning infrastructure	A	High
Rhode Island	$156.2M	no major differentiator surfaced in current sweep	C	Low
South Carolina	$200.0M	no major differentiator surfaced in current sweep	C	Low
South Dakota	$189.5M	no major differentiator surfaced in current sweep	C	Low
Tennessee	$206.9M	internal commercial anchor + strongest current proof story	A	High

State	FY26 RHT Award	Extra state-specific public signals surfaced in current sweep	Initial tier	Confidence
Texas	$281.3M	explicit RHT-style county outcome analog + CHNA relevance	A	High
Utah	$195.7M	no major differentiator surfaced in current sweep	C	Low
Vermont	$195.1M	extreme rurality; recent health-equity dashboards; EJ-tool development	B	Medium
Virginia	$189.5M	no major differentiator surfaced in current sweep	C	Low
Washington	$181.3M	official health disparities / equity tools and environmental justice materials	B	Medium
West Virginia	$199.5M	no major differentiator surfaced in current sweep	C	Low
Wisconsin	$203.7M	no major differentiator surfaced in current sweep	C	Low
Wyoming	$205.0M	no major differentiator surfaced in current sweep	C	Low

Recommended Use In Campaign Planning

- `Launch to all 50 states` remains the right top-of-funnel move.
- `Prioritize Tier A first` for research depth, Thad-led first-touch customization, and earliest Mike discovery participation.
- `Use Tier B as the fast-follower bench` for message-lane testing and second-wave discovery.
- `Keep Tier C in broad launch coverage and nurture` until either public evidence improves or real campaign behavior proves stronger receptivity than the current public record suggests.

What Still Needs More Research

The current ranking is materially better than the earlier one, but it is still limited by uneven public evidence across states. The biggest remaining gap is not national demand; it is `state-specific differentiation evidence` for middle-tier states.

Highest-value next research moves:

1. deepen the per-state review for Tier B and upper Tier C states with strong RHT awards but thin public-health analytics evidence
2. add a `commercial access path` overlay based on actual named contacts, partner routes, and existing LSA relationships
3. test campaign response data against this ranking and revise quarterly

Sources

- `docs/research/hsra/hsra-government-market-analysis-2026.md`
- `docs/research/market/hsra-demand-signals-rural-health-2026.md`
- `docs/research/hsra/chatgpt-research/chatgpt-deep-research-result-2026.md`
- `docs/strategic-plan/hsra/HSRA-tier-1-state-account-scorecard.md`
- https://www.cms.gov/newsroom/press-releases/cms-announces-50-billion-awards-strengthen-rural-health-all-50-states
- official state sources surfaced during targeted research for CT, KY, PA, NY, MN, WA, CO, MD, IL, NJ, and CA